HEDIS Quality Measure Automation

KANINI | Vertical Use Case Portfolio 2026

Use Case HC-02 | Healthcare — Payer Compliance & Quality | Tier 1 Strategic Imperative

Field	Value
Document Type	Vertical Use Case — Business Challenge & Market Assessment
Domain / Sub-domain	Healthcare (Payer) — Quality Measurement, HEDIS Reporting and Medicare Star Ratings
Target Geography	United States
Primary Economic Buyer	VP / Director of Quality & Stars; Chief Medical Officer; Chief Data / Information Officer (health plans, Medicaid MCOs, provider-sponsored plans)
Primary Users	HEDIS project managers, quality analysts, medical record abstractors (RN/coders), outreach and provider-engagement coordinators
Industry Spend Pool	Medicare Advantage quality bonus payments of at least $13.4B in 2026, up from $12.7B in 2025 [2][3]. Administrative cost of quality measurement is not reliably published (BC06's "$4.5B" figure could not be verified; see 1.4)
Addressable Market	US payer HEDIS/Stars software plus chart retrieval and abstraction services: triangulated at roughly $1.0B–$2.0B (2025), growing ~10–14% CAGR; published estimates range widely [29][30][31][32]
Existing KANINI Material	`BC06_HEDIS_Automation _v_1.0 4.pdf` (KodeGenAI 2026 hackathon challenge, June 2026). This document is based on it; measure references are corrected and figures re-verified
Demo Data Basis	Synthea synthetic FHIR R4/CSV patients and claims, NLM VSAC value sets, CMS eCQM/dQM CQL libraries, CMS Star Ratings cut points (Appendix A)
Research Confidence	Medium. Regulatory and CMS/NCQA facts are High (primary sources). Market sizing is Low–Medium (paywalled, low-transparency reports). Competitor weaknesses are Medium–Low (KLAS detail paywalled)
Version	1.0 — September 2026

Executive Summary

HEDIS is the quality-measurement backbone of US managed care. NCQA maintains 90+ measures, and more than 235 million people are enrolled in plans that report them [1]. HEDIS is also the largest data source behind Medicare Advantage Star Ratings, which drove at least $13.4B in quality bonus payments in 2026 [2]. Most plans still run HEDIS as a spring, backward-looking batch. It depends on costly medical-record "chart chase" for hybrid measures, and gaps are often found only after the measurement year closes. Three things make this urgent. NCQA will retire the hybrid method by MY 2029 and move to digital (FHIR + CQL) HEDIS by about 2030 [9][10][11]. Stars methodology is unstable: 11 measures are being removed, the equity reward was dropped, and a May 2026 court ruling limited Stars data to HEDIS, CAHPS and HOS [5][21]. And CMS-0057-F FHIR APIs go live in January 2027 [17]. The market is crowded. About 80 organizations appear on NCQA's measure-certification status list [15], and three different vendors won Best in KLAS in 2024–2026 [33][34][35]. The real gap is narrower: helping mid-sized and Medicaid plans become digital-HEDIS-ready and replace hybrid chart chase with year-round, auditable supplemental data.

1. The Business Problem

1.1 Problem Statement

HEDIS (Healthcare Effectiveness Data and Information Set) is the most widely used performance-measurement framework in US managed care. NCQA (National Committee for Quality Assurance) develops and maintains it. It has more than 90 measures across six domains: Effectiveness of Care; Access/Availability of Care; Experience of Care; Utilization and Risk-Adjusted Utilization; Health Plan Descriptive Information; and Measures Reported Using Electronic Clinical Data Systems [1]. Commercial, Medicare and Medicaid plans report HEDIS for NCQA accreditation, NCQA Health Plan Ratings, state Medicaid contracts and, most importantly, Medicare Advantage (MA) Star Ratings. In MY 2023, NCQA received HEDIS submissions from 420 commercial, 278 Medicaid and 760 Medicare plan reporting units, 1,458 in total [14].

The money involved is large. MA contracts rated 4 stars or higher get a 5-percentage-point benchmark increase (10 points in "double bonus" counties). They also keep a larger share of the gap between benchmark and bid as rebates: 65–70% versus 50% for contracts below 3.5 stars [3]. Federal spending on the quality bonus program reached at least $13.4B in 2026, up from $12.7B in 2025 and more than four times the 2015 level. About 68% of the 35 million MA enrollees are in bonus-qualifying plans [2]. HEDIS measures are the largest block of clinical-process and outcome measures in the Part C Star Rating, about 25% of the overall rating weight for MY 2026 [62]. That share will matter more after the 2026 court ruling that CMS may base Star Ratings only on HEDIS, CAHPS and HOS data (Section 1.3) [21].

Even so, many plans still run HEDIS as an annual, retrospective batch. The measurement year closes on December 31. Plans then chase medical records for hybrid measures from January to early May, finish abstraction by early May, and submit to NCQA by mid-June [28]. By the time a plan finds that a member with diabetes had no glycemic-status test (GSD measure), or that a hypertensive member's last blood pressure was 148/92 (CBP measure), the year is over. The quality score, the bonus money and the chance to get the member recommended care are all lost.

Chart chase is the main cost driver. For hybrid measures, plans draw a systematic sample (historically 411 members per measure, with oversample) and retrieve records from provider offices [65]. Large plans also run separate retrieval programs for risk adjustment. Datavant describes one plan initiative that retrieved 2 million charts across 1.5 million members (vendor-reported) [26]. Public, neutral data on the per-chart cost of payer HEDIS retrieval is scarce. The $15–$50 per chart range used in BC06 is a common industry rule of thumb, but this research could not verify it. What is published: the HIPAA flat fee for an electronic patient-requested copy can be up to $6.50, and state fee schedules for other requesters run to caps of $80–$150+ [27]. The real cost is the retrieval vendor fee plus the labor of credentialed abstractors, which this research could not quantify from neutral sources.

1.2 Why This Problem Persists

- **Measure logic is complex and changes every year.** Each measure has an initial population (eligible population), continuous-enrollment rules (typically 12 months with no more than one gap of up to 45 days), required and optional exclusions (hospice, palliative care, advanced illness and frailty for older members), and numerator logic across several code systems. NCQA changes measures annually. For MY 2026 it added 7 measures (four risk-adjusted post-surgical hospitalization measures, DDM, AAF-E and a tobacco-cessation ECDS measure), retired 2 (Asthma Medication Ratio and Medical Assistance With Smoking and Tobacco Use Cessation), moved four measures to ECDS (LSC, SPC, SPD, and BPD-E voluntarily), and reissued the whole specification in a FHIR-aligned format [8]. In the MY 2027 cycle, NCQA announced that from MY 2028 it will stop excluding denied claims in 21 measures [10]. Every change means re-engineering the logic and re-validating it.
- **The measures BC06 named have themselves changed.** The old Comprehensive Diabetes Care (CDC) composite no longer exists. Diabetes care is now measured as separate measures: Glycemic Status Assessment for Patients With Diabetes (GSD, which accepts HbA1c or GMI), Eye Exam for Patients With Diabetes (EED), Blood Pressure Control for Patients With Diabetes (BPD) and Kidney Health Evaluation for Patients With Diabetes (KED) [9][4]. Adult BMI Assessment (ABA) has been retired. Breast, colorectal and cervical cancer screening are now ECDS measures (BCS-E,

COL-E, CCS-E) [14]. The fact that an internal KANINI document from June 2026 was already out of date shows how quickly this domain moves.

- **Data is fragmented, and much of it sits outside claims.** One measure can need medical and pharmacy claims, LOINC-coded lab results, vital signs from EHRs, HIE feeds, case-management registries and provider-submitted supplemental files. NCQA's ECDS data hierarchy ranks EHR data first, then HIE/registry, then case management, then claims [14]. In a 2025 Arcadia survey, 85% of health plans had not yet centralized their data across systems (vendor-reported) [44].
- **Annual batch processing finds gaps too late.** A gap found in March for the prior year cannot be closed. Prospective, year-round review is spreading. For example, Astrata sells "year-round prospective HEDIS" chart abstraction for whole populations [39]. But many plans still depend mostly on the retrospective season.
- **Hybrid chart chase does not scale, and it is going away.** The hybrid method exists because some clinical facts (a BP reading, a retinal exam result) were missing from administrative data. NCQA will remove it by MY 2029 [9][10]. Once a measure is ECDS-only, a chart found in April no longer counts. Evidence must reach the plan as supplemental data during the measurement period. One vendor estimates this needs "roughly 35x to 75x more supplemental data per measure" (vendor-reported, not independently verified) [58].
- **The rules around the scores keep moving.** Tukey outlier deletion in cut-point setting, first used for the 2024 Star Ratings, pushed many cut points up. Earlier CMS modeling had suggested it would lower 16% of contracts by half a star [64]. The weight of patient-experience measures dropped from 4 to 2 for 2026 [4]. Eleven measures are being removed from Stars for 2028–2029 [5][7]. The Health Equity Index / "Excellent Health Outcomes for All" reward was cancelled before its first use [5][63]. Plans find it hard to forecast which gaps are worth money.
- **The economics favor incumbents and discourage rip-and-replace.** Measure logic must be NCQA-certified, and results must pass a HEDIS Compliance Audit by an NCQA-licensed organization. Switching engines in the middle of a season is risky. So plans stay with incumbents even when those tools are slow or opaque. One consultancy estimates that building an in-house validated digital engine costs about $1.15M a year fully loaded (not independently verified) [59].

1.3 Rationale and Industry Context

- **Bonus dollars are at a record high, but fewer plans get them.** QBP spending reached at least $13.4B in 2026 [2]. Yet only 207 MA-PD contracts (about 40%) earned 4+ stars for 2026, covering about 64% of MA-PD enrollees. The enrollment-weighted average rating was 3.98, compared with 4.14 in 2023 [4]. Of 516 rated contracts, 175 sit at 3.5 stars, one half-star below the bonus threshold [4]. For these contracts, closing HEDIS gaps is the most direct lever they control.
- **Stars litigation has made HEDIS matter more.** Humana's challenge over call-center test calls was dismissed with prejudice in October 2025. Analysts estimated the downgrade could cost Humana more than $1B a year, and the share of its members in 4+ star plans fell to 20% for 2026 [20]. In *Clover Insurance Co. v. HHS* (S.D. Ga., May 27, 2026), the court held that CMS may use only the data types in 42 U.S.C. § 1395w-22(e) (HEDIS, CAHPS and HOS). It invalidated ten measures, including the Part D medication-adherence measures and call-center measures, and raised Clover's rating from 3.5 to 4.5 stars, worth about $120M in bonus payments [21]. On June 17, 2026, CMS said it would voluntarily recalculate ratings for every MA organization on that basis for 2027 QBP [21]. Elevance ($115M claimed), Alignment and SCAN then sued over the recalculations [22][23]. If the ruling stands, a larger share of each contract's rating will rest on HEDIS.
- **The CY 2027 MA/Part D final rule (April 2026) shifts Stars toward clinical measures.** CMS is removing 11 measures focused on administrative processes, cancelling the Excellent Health Outcomes for All reward and keeping the historical reward factor, and adding a Part C Depression Screening and Follow-Up measure starting with MY 2027 (2029 Star Ratings) [5][6]. Removals include Statin Therapy for Patients with Cardiovascular Disease (from 2028 Stars) and several complaint, appeals and customer-service measures (from 2029 Stars). Diabetes Care – Eye Exam was proposed for removal but retained [7].
- **NCQA has set dates for digital HEDIS.** NCQA plans fully digital HEDIS by about MY 2030 and removal of the hybrid method by MY 2029 [9][11]. The February 2026 update changed some measure-by-measure paths. GSD adds optional

ECDS reporting in MY 2027 and becomes ECDS-only by MY 2029. WCC is to be retired in MY 2029. PPC gets an ECDS risk-based replacement by MY 2028. TRC and COA get ECDS versions in MY 2028 [10]. CBP and BPD were originally scheduled to move to ECDS in MY 2028 [9]. NCQA delivers digital measures as CQL through its Digital Content Services (launched 2023). Licensees must supply FHIR data mapped to the HEDIS Core FHIR Implementation Guide [13]. NCQA also published open-source CQL engine requirements and software through Firely [12]. As of March 2026, NCQA no longer requires a formal IDSS parallel-reporting year before digital submission, although any CQL engine used must be certified [11].

- **CMS is moving all of its quality programs to digital measures.** CMS's Digital Quality Measurement strategy aims to convert all measures in CMS programs to FHIR-based dQMs. A July 2025 update, "Charting a Path Forward: Transition to Digital Quality Management", and January 2026 education materials on moving from the eCQM approach to FHIR-based dQMs followed. FY 2026 payment rules carried "Toward Digital Quality Measurement" provisions [16].
- **CMS-0057-F builds the FHIR data pipes that digital HEDIS needs.** MA organizations, Medicaid and CHIP programs and managed care plans, and FFE QHP issuers must implement Provider Access, Payer-to-Payer and Prior Authorization FHIR R4 APIs by January 1, 2027. These APIs share USCDI clinical data plus claims and encounter data [17]. The same normalized FHIR data a plan builds for these APIs is the input a digital HEDIS engine needs.
- **Medicaid quality reporting is now mandatory.** Reporting the Child Core Set and the behavioral health measures of the Adult Core Set became mandatory for all states and territories in 2024 [18]. The 2023 final rule phases in stratified reporting, starting with 25% of mandatory measures for 2025 and 50% for 2026 [19]. Many Core Set measures are HEDIS-derived, and states pass the reporting burden down to Medicaid MCOs.
- **Value-based contracts depend on HEDIS-style measures.** Risk-bearing provider groups, ACOs and IPAs are paid on HEDIS-like gap closure across several payer contracts. This is why vendors such as Cozeva (25M+ lives, 84 clients, vendor-reported) sell into both plans and provider organizations [51], and why Innovaccer and Arcadia sell one quality platform to payers and providers [43][45].

1.4 Scale of the Problem (Key Metrics)

Metric	Value	Source
HEDIS measures / domains	More than 90 measures across 6 domains	[1]
People in plans reporting HEDIS	More than 235 million	[1]
HEDIS plan submissions to NCQA (MY 2023)	1,458 (420 commercial, 278 Medicaid, 760 Medicare)	[14]
MA quality bonus payments	At least $13.4B (2026); $12.7B (2025)	[2][3]
MA enrollees in bonus-qualifying plans	68% of about 35M (2026), down from 75% (2025)	[2]
Average annual bonus per enrollee in bonus plans	$381 (individual), $466 (employer), $318 (SNP)	[2]
MA-PD contracts at 4+ stars (2026 Stars)	207 of 516 (about 40%); about 64% of enrollees	[4]
MA-PD contracts at exactly 3.5 stars (2026)	175 (33.9%), one half-star below the bonus	[4]
Value of a half-star at the 4.0 threshold (500K-member plan)	About $190M–$233M/year in bonus alone. KANINI derivation: 500K × $381–$466 [2]; excludes rebate and enrollment effects	Derived from [2]
Example half-star disputes	Clover 3.5→4.5 recalculation, about $120M; Elevance claim, $115M; Humana downgrade estimated at over $1B/year	[20][21] [22]

Metric	Value	Source
HEDIS share of Part C Star Rating weight (MY 2026)	About 25%	[62]
Hybrid method end / digital-only target	Hybrid removed by MY 2029; fully digital about MY 2030	[9][10][11]
Plans submitting ECDS results (MY 2023)	100% commercial, 96.8% Medicaid, 95.1% Medicare	[14]
Organizations on NCQA MY 2025 measure-certification status list	About 80 (certified or seeking)	[15]
Chart chase cost per chart	$15–$50 (BC06 rule of thumb, **not verified**). Reference points: $6.50 HIPAA electronic flat fee; state caps $80–$150+	[27]
Cost of quality reporting, provider side	$15.4B/year across four physician specialties (2016, **dated**); one academic hospital: about $5.0M personnel plus $0.6M vendor fees and 108,478 hours for 162 metrics	[24][25]
BC06 "$4.5B annual quality measurement program spend"	**Not verified**. No primary source found; not used in this document	—

2. High-Level Objective

2.1 Solution Vision

The application runs HEDIS continuously instead of once a year. It ingests claims, pharmacy, lab, EHR and supplemental data into a FHIR-normalized member record. It computes HEDIS measures (traditional and digital CQL) on a weekly or daily cadence during the measurement period. It turns each open gap into a specific, evidence-backed closure action for outreach teams, providers and abstractors. It also:

- Pulls structured evidence out of unstructured charts (the work the hybrid chart chase does today) year-round, so evidence counts as supplemental data before the ECDS-only deadline, not as retrospective abstraction afterward.
- Estimates what each gap is worth for Star Ratings, NCQA Health Plan Ratings, Medicaid quality withholds and value-based contracts, and ranks work by value and likelihood of closure.
- Prepares the plan for NCQA's digital-only HEDIS by running the FHIR/CQL pipeline next to the incumbent certified engine and reconciling the two at member level.

Human-in-the-loop stance. HEDIS is a regulated, audited submission, so the system does not decide compliance for evidence taken from unstructured records. AI proposes. A credentialed abstractor or quality analyst confirms every chart-derived numerator hit or exclusion before it enters the supplemental data file, and every decision has a traceable audit trail for the HEDIS Compliance Audit. Deterministic measure logic (certified or validated) computes the rates. AI assists with evidence retrieval, prioritization and anomaly detection, and never changes the measure specification.

2.2 Solution Approach: Deterministic Digital Measure Engine + AI-Assisted Evidence Abstraction and Gap Prioritization

Layer	What It Does	Examples
Data Ingestion and FHIR Normalization	Maps claims (837/encounter), pharmacy (NCPDP), lab (LOINC), EHR/HIE (C-CDA, FHIR), and CMS-0057-F Provider Access / Payer-to-Payer data to FHIR R4 resources that match NCQA's HEDIS Core FHIR IG	Claim line with CPT 45378 (colonoscopy) → FHIR `Procedure`. Lab result LOINC 4548-4 (HbA1c) = 7.4% → `Observation`. Enrollment spans → `Coverage` with gap detection
Value Set and Measure Library	Keeps versioned value sets and measure logic per measurement year (MY 2026, MY 2027), with traditional specs and NCQA digital CQL side by side	GSD numerator uses HbA1c LOINC codes such as 4548-4, 4549-2 and 17856-6, plus GMI. Diabetes denominator uses ICD-10 E10.x/E11.x/E13.x. CBP uses systolic LOINC 8480-6 and diastolic 8462-4, and CPT II 3074F/3075F and 3078F/3079F
Measure Calculator	Runs initial population, continuous enrollment, exclusions and numerator for every member, and produces rates, member-level detail and an audit trail. Either a certified engine (licensed) or an open-source CQL engine validated against NCQA test decks	**GSD, <8.0% indicator:** members 18–75 with diabetes; most recent glycemic status in the measurement period below 8.0%; exclusions for hospice, palliative care (e.g., ICD-10 Z51.5) and advanced illness with frailty (66+). **COL-E:** members 45–75; colonoscopy in the period or 9 prior years, FIT (CPT 82274) in the period, or stool-DNA (CPT 81528) within 3 years
Prospective Gap Identifier	For each member in the denominator who does not yet meet the numerator, generates a gap with the exact missing element, the latest relevant event and the deadline	Member A (GSD): last HbA1c 9.2% on 2026-03-11, so **not controlled**. Action: repeat A1c after therapy change before 12/31. Member B (CBP): BP 146/88 on 2026-07-02, so **not controlled**. Action: schedule recheck; a later reading below 140/90 closes the gap because the *most recent* reading counts
AI-Assisted Evidence Abstraction	Uses NLP/LLM extraction on retrieved charts, provider portal uploads and HIE documents to find evidence that closes a gap or supports an exclusion, citing the page and passage, and sends it to an abstractor queue	Finds "Colonoscopy 2019, Dr. X, no polyps" in a 2026 PCP note, closing COL-E as documented history. Finds "BP 132/78" in a scanned vitals flowsheet dated 2026-09-14. Finds a frailty diagnosis supporting an exclusion. Each item is **proposed**, and an abstractor approves it as non-standard supplemental data

Layer	What It Does	Examples
AI Gap Prioritization	Scores gaps by likelihood of closure and financial and Stars value, using engagement history, scheduled visits, provider attribution and measure weight	Member A has a PCP visit in 9 days at a high-closure practice, and GSD is a triple-weighted outcome measure, so **priority HIGH**. Member C has no contact in 18 months, no attributed PCP and falls in a low-weight measure, so **priority LOW**, routed to a mailed FIT kit campaign
Stars and Contract Impact Modeler	Projects measure stars against prior-year cut points and scenario bands (Tukey, guardrails, measure removals) and converts gap closure into rating and bonus sensitivity	"Closing 1,150 more GSD gaps moves the projected rate from 72.1% to 76.4%, crossing the prior-year 4-star cut point." Results are shown as a range, since cut points are only known after the fact
Data Quality and Anomaly Detection	Flags feeds and mappings that silently drop numerator hits	"Lab feed from Vendor Q has shown 0 HbA1c results since week 31." "38% of BP observations missing units." "Denominator down 6% after enrollment file change"

2.3 How the System Would Work (Functional Flow)

From the point of view of a health plan HEDIS / Stars quality manager:

1. The quality manager picks the measurement period (e.g., MY 2026) and the programs to track: MA Stars, NCQA Health Plan Ratings, Medicaid Core Set / state withhold, and provider value-based contracts.
2. The system loads the latest feeds (claims, pharmacy, lab, EHR/HIE, supplemental files, CMS-0057-F API data), normalizes them to FHIR, and shows a data-completeness scorecard by source and measure.
3. The system computes all selected measures and shows, for each one, the current rate, denominator, numerator, exclusions, open gaps, the prior-year cut points, and projected year-end rate at current closure velocity.
4. The quality manager drills into a measure (e.g., CBP), sees the member-level gap list, and filters it by attributed provider group, line of business or priority.
5. For each gap, the system shows the missing element, the last relevant event (date, value, source), the recommended action, and any AI-found evidence waiting for abstractor review.
6. Abstractors work a queue of AI-proposed evidence, viewing the highlighted source passage and accepting or rejecting each item. Accepted items become supplemental data with full lineage.
7. Outreach and provider-engagement teams get prioritized worklists (member outreach, provider gap reports, point-of-care alerts). Closures come back as new claims, results or supplemental data.
8. Each week, the system reconciles its results against the incumbent certified engine (traditional versus digital), flags differences at member level, and records a digital-readiness score by measure.
9. At season end, the system produces audit-ready supplemental data files, the reconciliation evidence and documentation for the HEDIS Compliance Audit. Submission itself goes through the plan's certified engine and NCQA IDSS.

2.4 What Success Looks Like

Success Metric	Target	Why It Matters
Measure-logic accuracy	100% agreement with NCQA-style test deck cases; production rates within ±0.5 percentage points of the incumbent certified engine on parallel runs	Any error moves Star cut-point outcomes and fails audit. BC06's "within 2%" is too loose for a regulated submission
In-year gap visibility	Gap lists refreshed at least weekly from month 2 of the measurement period (baseline: annual or quarterly)	Gaps must be seen while they can still be closed
Prospective gap closure	+5 to +10 percentage-point improvement in gap-closure rate on targeted measures versus prior year (baseline: plan's MY rate)	Directly moves measure stars and bonus eligibility

Success Metric	Target	Why It Matters
Hybrid chart-chase volume	30–50% fewer retrospective chases on hybrid measures by capturing evidence prospectively (baseline: prior-year chase count)	Cuts retrieval and abstraction cost and prepares for MY 2029 hybrid removal
Abstractor productivity	2–3x more gaps reviewed per abstractor-hour with AI-proposed evidence (baseline: manual review). Vendor claims run far higher [39]	Credentialed abstractors are the scarce, costly resource
AI evidence precision (abstractor acceptance rate)	At least 90% of proposed numerator evidence accepted by abstractors; zero unreviewed AI items in submission files	Protects audit integrity and abstractor trust
Digital HEDIS readiness	At least 80% of in-scope measures reconciled traditional-versus-digital within tolerance before the plan's digital cut-over year	NCQA digital-only by about 2030 [11]
Stars projection error	Projected measure star equals actual in at least 80% of measures (± half-star at overall level)	Leaders move budget according to projections
Data-feed anomaly detection	Material feed drop-outs detected within 7 days	Silent feed failures are a common, avoidable cause of rate drops
Full-population compute time	Under 60 minutes for 1M members across 20+ measures	Makes daily or weekly refresh possible

3. Market Analysis

3.1 Market Size and Growth

No neutral analyst publishes a clean "US payer HEDIS/Stars software" market. Available estimates use different scopes and should be read as a range, not a point:

Estimate	Scope	Value	Growth	Source
HEDIS Reporting Solutions (global)	HEDIS software + services; North America >48%	$1.2B (2024) → $3.4B (2033)	12.1% CAGR	Market Intelo [30] (low-transparency publisher)
Healthcare Quality Management (global)	Provider-heavy QMS incl. document mgmt, analytics	$1.26B (2024) → $2.51B (2030)	12.1% CAGR	MarketsandMarkets, Oct 2025 [29]
Healthcare Quality Management Software (global)	Software only	$1.74B (2025) → $3.32B (2030)	13.9% CAGR	The Business Research Company [31]
Medical Records Retrieval (global)	All requesters (payer, legal, life insurance); North America ~45%	$1.1B (2024) → $2.8B (2034)	10.1% CAGR	Business Research Insights [32]

KANINI triangulation (US, payer HEDIS/Stars). Taking the US share of the HEDIS-specific estimate (about 48% of $1.2B) gives about $0.6B for software and services [30]. Retrieval and abstraction for quality (a fraction of the about $0.5B North American retrieval market [32], which is dominated by risk adjustment) and the payer share of quality-management analytics [29][31] add to that. A bottom-up check: 1,458 HEDIS reporting submissions [14] at an assumed blended $0.5M–$1.2M per year each (engine licence, analytics, retrieval, abstraction and outside services) gives about $0.7B–$1.75B. **Working estimate: US SAM of about $1.0B–$2.0B, growing about 10–14% a year.** The spread is wide because vendors bundle HEDIS with risk adjustment, retrieval and Stars consulting, and published reports do not break out payer quality. The per-submission spend assumption has not been validated with buyers (see 5.4).

The **value at stake** for buyers is much larger than their software spend: at least $13.4B a year in MA bonuses [2], plus rebate effects, Medicaid quality withholds and value-based contract incentives. That gap between software cost and value at stake is why plans pay for software, retrieval and consulting at the same time.

3.2 Buyer Segments and Buying Dynamics

Segment	Approx. size / examples	Buying behavior
National MA carriers	UnitedHealthcare, Humana, CVS/Aetna, Elevance, Centene, Kaiser. Most MA enrollment	Large in-house quality-engineering and Stars teams. Multi-vendor stacks (certified engine + retrieval vendor + in-house analytics). Buy point capabilities (AI abstraction, digital pipeline) rather than platforms. Long procurement, heavy security review
Regional, Blue and provider-sponsored plans	Hundreds of reporting units in the 760 Medicare and 420 commercial submissions [14]	Usually outsource the engine to a certified vendor (Inovalon, Cotiviti, SS&C, HealthEdge, etc.). Thin IT teams. Most exposed to the half-star threshold (the 3.5-star cluster of 175 contracts [4]). **Best fit for a digital-readiness and prospective-evidence offer**
Medicaid MCOs	278 Medicaid submissions [14]; state Core Set and withhold programs [18][19]	Cost-sensitive and driven by state contracts. Heavy hybrid reliance for prenatal/postpartum (PPC) and pediatric measures, which are among those changing next [10]
Risk-bearing providers / ACOs / IPAs	Contracted on HEDIS-like gaps across many payers	Want one multi-payer gap view and point-of-care workflow. Buy from Cozeva, Innovaccer, Arcadia, Pelica-type vendors [43][45][51][58]
State Medicaid agencies	50 states + DC and territories (mandatory Core Set)	Buy through state procurement or EQRO arrangements. Slow, RFP-driven

- **Economic buyer and user.** The VP of Quality/Stars or the CMO owns the budget and the Stars P&L. The CIO/CDO owns data and integration. Users are HEDIS analysts, abstractors, outreach coordinators and provider-network staff. Compliance and audit teams (and the external HEDIS auditor) effectively hold a veto.
- **Sales cycle and calendar.** Buying follows the HEDIS season. Decisions are usually made in summer and fall so a vendor can be live for the next measurement year before the January–June retrospective season. Engine switches mid-season are very rare. Vendors describe enterprise deployments as "multi-quarter" [58].
- **Build versus buy.** Nearly every plan buys certified measure logic, because certification and annual revalidation are costly (about $1.15M a year to build and maintain in-house, per one consultancy, not independently verified) [59]. The same source suggests plans below about 500,000 MA members are better served by validated vendor engines [59]. Plans do build their own data platforms, outreach analytics and Stars modeling, and that is where systems integrators compete.
- **Procurement realities.** Buyers require HITRUST/SOC 2, BAAs, PHI handling, NCQA certification evidence and auditor acceptance. They need references from similar plans. Pricing is rarely published and is typically per-member-per-month or enterprise subscription (industry practice; no vendor publishes rates).

3.3 Market Drivers and Headwinds

Driver / Headwind	Type (Regulatory / Economic / Technology / Labour)	Impact	Source
NCQA removes hybrid method by MY 2029; digital HEDIS about 2030	Regulatory	**Strong driver.** Forces FHIR pipelines and year-round supplemental data; changes retrieval economics	[9][10][11]
MA QBP at a record $13.4B while fewer contracts qualify (68% of enrollees, down from 75%)	Economic	**Strong driver.** Half-star moves are worth hundreds of millions for large plans	[2][4]
Clover ruling limits Stars data to HEDIS/CAHPS/HOS; CMS recalculation June 2026	Regulatory / Legal	**Driver, with uncertainty.** Raises HEDIS weight, but appeals and further litigation could change the method again	[21][22][23]
CY 2027 final rule removes 11 measures, cancels EHO4All reward, adds depression screening	Regulatory	**Driver.** Shifts Stars toward clinical measures. Removes the equity-stratified analytics use case plans had prepared for	[5][6][7]
CMS-0057-F FHIR APIs due January 1, 2027	Regulatory / Technology	**Driver.** Plans are building FHIR data pipes anyway, which digital HEDIS can reuse	[17]
Mandatory Medicaid Core Set reporting (2024+) and stratification phase-in	Regulatory	**Driver** for Medicaid MCOs and states	[18][19]
NCQA open-sourced CQL engine requirements; Digital Content Services	Technology	**Driver.** Lowers entry barriers for digital engines and makes logic less proprietary	[12][13]
NCQA removes denied-claims exclusion from 21 measures (MY 2028)	Regulatory	Neutral to driver. More logic re-engineering; rates may shift	[10]
Crowded vendor field (~80 organizations on NCQA list; rotating Best in KLAS)	Economic	**Headwind** for new entrants. Price pressure and incumbent lock-in	[15][33][34][35]
Stars methodology volatility (Tukey, weight changes, litigation)	Regulatory	Mixed. Drives demand for modeling, but plans hesitate to commit to long programs	[4][64]
Scarcity and cost of credentialed abstractors and quality staff	Labour	Driver for AI-assisted abstraction. No neutral statistic located for 2025–2026	— (not verified)
Provider abrasion from duplicative record requests	Labour / Economic	Driver for member-based and digital retrieval	[26]

Driver / Headwind	Type (Regulatory / Economic / Technology / Labour)	Impact	Source
AI in quality workflows faces audit acceptability questions	Regulatory / Technology	**Headwind.** AI-found evidence must meet NCQA audit standards for non-standard supplemental data	[11] (engine certification requirement)

3.4 Technology and Adoption Trends

- **ECDS is now mainstream.** In MY 2023, 100% of commercial, 96.8% of Medicaid and 95.1% of Medicare plans submitted ECDS-based results, up from 50.0% of Medicare plans in MY 2021. NCQA found that plans using non-claims electronic data generally performed better than claims-only plans [14]. MY 2026 is the first full Stars year with Colorectal Cancer Screening as ECDS-only (vendor-reported) [50].
- **Digital engine validation has started.** Several vendors now market NCQA-validated FHIR/CQL engines. Examples include Astrata's eMeasure ("one of the first NCQA certified digital HEDIS engines") [39], Smile Digital Health's OmniQ dQM (80 measures, "5 billion data points" in under 12.5 hours, vendor-reported) [41], and Cozeva's "native FHIR digital measure engines" [51]. A competitor-authored comparison reports that Cotiviti's Quality Intelligence was certified in June 2026 for all available NCQA digital HEDIS measures, and that Inovalon claims 100% validation coverage for digital measures. Neither claim was verified from primary sources [58].
- **AI abstraction is common among retrieval and abstraction vendors.** Astrata claims abstractors work "760% faster" with NLP-assisted chart review [39]. MRO launched its Prodigy AI abstraction engine in September 2026 [53]. Datavant acquired AI record-analysis company DigitalOwl in 2025 [47]. Reveleer markets AI-enabled retrieval for HEDIS [38]. All of these figures are vendor-reported.
- **Prospective and point-of-care models are growing.** Examples: Innovaccer's EHR-embedded InNote and pharmacy-based gap closure through EQuIPP [45]; Cozeva's merger with Vatica Health, a point-of-care risk and quality company ranked #1 by KLAS for three years through 2025 (vendor-reported) [51][52]; Reveleer's bi-directional HIE/EHR integration [38].
- **Analyst views.** KLAS rates the "Quality Measurement & Reporting (Payer)" segment. The Best in KLAS winner changed every year: Cotiviti Quality Intelligence in 2024 (83.8), Cozeva PayerOne Quality in 2025 (88.1) and Inovalon Converged Quality in 2026 (88.6) [33][34][35]. This suggests close competition and no dominant leader on customer satisfaction. IDC's 2025 MarketScape for US Value-Based Healthcare Analytics evaluates HEDIS/Stars reporting modules as core capabilities [66]. Everest Group's 2025 payer care-management assessment names Stars optimization as a differentiator [67]. Detailed KLAS scores and interview commentary are paywalled.
- **AI adoption in decision-making is still early.** In Arcadia's 2025 survey, only 14% of healthcare leaders had fully integrated AI into decision-making (vendor-reported) [44].

4. Competitive Landscape

4.1 Competitor Categories

1. **Full-suite certified HEDIS / Stars platforms (incumbents):** Inovalon, Cotiviti (now including Edifecs), SS&C Health, HealthEdge (Quality360), Optum, Veradigm Payer Analytics, and many smaller certified engines on NCQA's list (e.g., MedeAnalytics, Milliman MedInsight, Gainwell, ZeOmega) [15].
2. **Payer-provider data and value-based-care platforms:** Innovaccer, Arcadia, Cozeva (+Vatica), Lumeris, Health Catalyst. They combine a certified engine with an aggregation, provider-workflow and analytics layer.
3. **Retrieval and abstraction specialists (services and tech-enabled):** Datavant (Ciox), Reveleer, MRO (merged with Q-Centrix), Optum/Episource, Cotiviti's retrieval services.
4. **Digital-quality and AI-native specialists:** Astrata (NLP abstraction + FHIR/CQL engine), Smile Digital Health (FHIR-native dQM engine), Pelica (AI "execution layer" for multi-payer gap closure).
5. **Provider-side quality reporting (adjacent, not payer HEDIS):** Medisolv (hospital eCQM, MIPS, ACO APP; no payer/HEDIS offering on its site) [54].
6. **In-house / DIY:** national carriers with their own quality engineering, often on an open-source CQL engine plus NCQA Digital Content Services [12][13], and systems integrators (e.g., CitiusTech, UST HealthProof, EXL, all on the NCQA list [15]).

4.2 Competitor Profiles

Pricing is not published by any vendor below. Weaknesses are drawn from available evidence. Where only competitor-authored or vendor sources exist, this is stated.

Company	Category	Target Segment	Core Offering for this Problem	Pricing Model (if public)	Strengths	Weaknesses / Complaints	Recent Moves (2024–2026)
Inovalon	Full-suite incumbent	Health plans of all sizes	Converged Quality: certified HEDIS engine and quality platform, on a large connected data network	Not public	2026 Best in KLAS, Quality Measurement & Reporting (Payer), score 88.6 [33]. 26th consecutive NCQA certification (per [58])	Characterized as measurement-focused, with member outreach left to plan teams; enterprise multi-quarter deployment (competitor-authored source [58]). Private (Nordic Capital / Insight Partners), so little disclosure [61]	New CEO January 2025 [61]. Best in KLAS 2026 [33]
Cotiviti (incl. Edifecs)	Full-suite incumbent + retrieval services	Health plans; large and regional	Quality Intelligence (certified engine, traditional + dQM, FHIR input), Star Intelligence, medical record retrieval and abstraction [36][37]	Not public	2024 Best in KLAS (83.8) [35]. More than 129M members reported to NCQA through its solution in MY 2024; retrieval rate above 90%; abstractor accuracy above 97% (vendor-reported) [36]	Outreach described as campaign-based rather than worked queues (competitor-authored [58]). Integrating Edifecs adds transition risk for customers (Edifecs customers told to keep working with existing contacts [37])	Edifecs now part of Cotiviti (acquisition reported March 2025 [58]; [37] confirms). Digital HEDIS certification reported June 2026 [58]

Company	Category	Target Segment	Core Offering for this Problem	Pricing Model (if public)	Strengths	Weaknesses / Complaints	Recent Moves (2024–2026)
Innovaccer	Payer-provider platform	Health systems, ACOs, payers	Quality Management on its data platform: end-to-end HEDIS (admin, hybrid, ECDS), chart-chase workflow, InNote point-of-care, pharmacy gap closure via EQuIPP [45]	Not public	NCQA certified (InGraph on MY 2025 list) [15]. Provider and pharmacy reach (95% of pharmacies via EQuIPP, vendor-reported) [45]. Well funded [46]	Broad "one-stop shop" strategy extending into care delivery (Story Health) may dilute payer-quality focus [46] (inference)	Acquired PQS/EQuIPP (March 2024), Humbi AI (January 2025), Story Health (September 2025); $275M Series F at about $3.45B valuation (January 2025) [46]
Arcadia	Payer-provider data platform	Payers, ACOs, provider groups	Quality management for payers: claims + clinical aggregation, gap identification, provider collaboration (Arcadia Assess), certified HEDIS measures [43]	Not public	NCQA HEDIS and Data Aggregator Validation certifications [43]. Claims about 0.3 Star points gained from data integration (vendor-reported, 2021 case) [43]	Described as analytics-focused rather than execution-oriented (competitor-authored [58]). Its public case evidence dates from 2021 [43]	Nordic Capital majority stake reported July 2025 (secondary source only [58]; **not verified**). 2025 partnerships with Surescripts, League, Quest Analytics [44]
Cozeva (+ Vatica Health)	Payer-provider platform + point-of-care services	Health plans and care delivery organizations	PayerOne Quality; NCQA-certified native FHIR digital measure engines; provider-workflow gap closure [51]	Not public	2025 Best in KLAS (88.1) [34]. 25M+ lives, 84 clients, 97K+ providers (vendor-reported) [51]	No independent weakness evidence located. Merger integration risk (inference)	Merged with Vatica Health (risk and quality point-of-care; KLAS #1 three years to 2025) [51] [52]. Merger date **not verified**
SS&C Health	Full-suite incumbent	Health plans, IHA AMP participants	CareAnalyzer HEDIS (with Johns Hopkins ACG), Star Ratings Analytics, AMP [42]	Not public	NCQA certification every year since 2002; "fully NCQA-certified HEDIS measures, including all ECDS measures" [42]	No specific public complaints located. Digital HEDIS materials are thought-leadership rather than a named certified dQM product [42]	Digital HEDIS transformation content (date not stated) [42]
HealthEdge (Quality360)	Full-suite incumbent (core admin adjacency)	Health plans (HealthRules/GuidingCare customers)	Quality360: HEDIS engine, abstraction, chase management, reporting (IDSS, PLD) [49]. Also delivered through UST HealthProof [50]	Not public	Integrated with core admin and care management. Claims 95%+ retrieval and a 28% Stars improvement in MY 2024 (vendor-reported) [49]	No independent weakness evidence located	Ownership change in 2025 **not verified** in this research
Optum (incl. Episource)	Full-suite + services	Health plans	Quality Management / EBM Connect certified measures [15]; retrieval services; Episource now redirects to Optum risk adjustment [60]	Not public	Scale; retrieval + coding + analytics under one roof [60]	Channel conflict: many plans are wary of sharing data with a UnitedHealth Group affiliate (market perception; **not verified** with survey data)	Episource integrated into Optum (episource.com redirects) [60]
Veradigm	Payer analytics (incumbent)	Health plans	Veradigm Payer Analytics (Pulse8 and Babel Health products, incl. Qualit8 quality) [55]; Veradigm Quality Analytics on NCQA list [15]	Not public	Provider EHR footprint plus payer analytics [55]	Corporate reporting and status issues since 2024 **not re-verified** in this research	Consolidated Pulse8 and Babel Health into Veradigm Payer Analytics [55]

Company	Category	Target Segment	Core Offering for this Problem	Pricing Model (if public)	Strengths	Weaknesses / Complaints	Recent Moves (2024–2026)
Reveleer	Retrieval / abstraction + AI	Health plans (MA, ACA, Medicaid)	AI-enabled retrieval and abstraction for HEDIS and risk; Care Gap Manager; point-of-care integration [38]	Not public	70+ health plan customers, 97% retention, 66M lives, 400M+ records a year (vendor-reported) [38]	Not a certified HEDIS engine vendor (not on NCQA MY 2025 list [15]). Described as less focused on the execution layer (competitor-authored [58])	Acquired Curation Health (October 2024) and Novillus (April 2025); launched EVE Hybrid AI (January 2026), all per a secondary source [58]
Datavant (Ciox)	Retrieval / data exchange	Health plans, providers, life sciences	Chart retrieval for risk and quality; digital exchange via TEFCA/FHIR; member-based retrieval [26][48]	Not public	Network of 80,000 hospitals and clinics; 75% of top-100 health systems (vendor-reported) [48]. About 80% of charts within 14 days on digital exchange projects (vendor-reported) [26]	Security incidents in 2021 and May 2024 [47]. Primarily risk-adjustment oriented [26]	Acquired Apixio Connected Care platform (2024), Aetion, Ontellus, DigitalOwl (2025) [47]
MRO (+ Q-Centrix)	Retrieval / abstraction (provider-anchored)	Providers, health plans, registries	Clinical data exchange, release of information, abstraction with "Collaborative Intelligence" [53]	Not public	12-time KLAS award winner (vendor-reported); 99%+ abstraction accuracy claim [53]	Provider/registry heritage; payer HEDIS depth unclear from public sources	Merged with Q-Centrix (date **not verified**); launched Prodigy AI abstraction engine September 2026 [53]
Astrata	Digital-quality / AI-native	Health plans	Year-round prospective HEDIS (NLP Chart Review), eMeasure FHIR/CQL engine, digital readiness assessments [39] [40]	Not public	Early NCQA-certified digital engine; claims 760% abstractor speed-up (vendor-reported) [39]	Small company; on NCQA list for a subset of measures (MY 2024 certified, MY 2025 seeking) [15]	Published "Path to Digital HEDIS by 2030" roadmap (March 2026) [40]
Smile Digital Health	Digital-quality specialist (FHIR platform)	Health plans, HIEs	OmniQ dQM – Measures for HEDIS: FHIR-native, NCQA-validated [41]	Not public	FHIR server heritage; 80 measures in under 12.5 hours (vendor-reported) [41]	No abstraction, retrieval or outreach. Needs a partner for the operational layer (inference from product scope [41])	Marketing the "end of traditional HEDIS" in 2026 [41]
Lumeris	VBC enablement	Provider-sponsored MA, ACOs	Operational Data Store and LumerisEngage (certified measures, MY 2025) [15]; Tom AI primary-care platform [56]	Not public	1.4M+ lives; $90B in claims ingested a year (vendor-reported) [56]	Quality measurement is a component of a broader care-model offering, not a standalone product [56]	Repositioned on "Tom" AI Primary Care as a Service [56]
Health Catalyst	Analytics platform	Health systems, some payers	Quality analytics; KPI Ninja and Able Health appear on NCQA list [15][57]	Not public	1,100+ organizations (vendor-reported) [57]	Provider-centric. HEDIS certification is for a subset of measures [15]	No 2025–2026 quality moves verified
Pelica Health	AI-native execution layer	IPAs, provider groups, plans	"Quality & Stars copilot": multi-payer gap closure, outreach, EMR overlays [58]	Not public	2–4 week deployment claim; 41% gap-closure improvement at a 200K-patient IPA (vendor-reported) [58]	Early-stage; no independent validation located	Published competitive comparison (June 2026) [58]

Company	Category	Target Segment	Core Offering for this Problem	Pricing Model (if public)	Strengths	Weaknesses / Complaints	Recent Moves (2024–2026)
Medisolv	Provider-side (adjacent)	Hospitals, clinicians, ACOs	eCQM, MIPS, ACO APP, AI registry abstraction [54]	Not public	Specialist in provider eCQM/dQM	No payer HEDIS offering [54]	AI registry abstraction [54]

4.3 Capability Comparison Matrix

● full / ◐ partial / ○ none / ? unknown (based on public evidence only)

Capability	Inovalon	Cotiviti	Innovaccer	Arcadia	Cozeva	Reveleer	Astrata	Smile
NCQA-certified traditional HEDIS engine	●	●	●	●	●	○	◐	○
Digital (FHIR + CQL) HEDIS engine validated	◐ (claimed [58])	◐ (claimed [58])	?	?	●	○	●	●
Medical record retrieval	?	●	◐	?	?	●	○	○
AI/NLP-assisted abstraction	?	◐	◐	◐	?	●	●	○
Year-round prospective gap closure	◐	◐	●	●	●	●	●	◐
Provider point-of-care workflow	?	?	●	●	●	●	?	○
Member outreach orchestration	○	◐	●	?	?	?	○	○
Stars / cut-point forecasting	?	●	?	◐	?	?	?	○
Mid-market speed of deployment (weeks, not quarters)	○	○	○	○	?	?	?	?

The matrix mostly shows that little of the landscape can be verified in public. KLAS detail and most feature documentation are gated. A buyer-side validation (Section 5.4) would turn many "?" cells into facts.

4.4 Market Consolidation and Funding Signals

Date	Event	Signal	Source
January 2024	Innovaccer acquires Cured (healthcare CRM)	Platform expansion into engagement	[46]
March 2024	Innovaccer acquires Pharmacy Quality Solutions (EQuIPP)	Pharmacy channel for gap closure and Part D quality	[46]
2024	Datavant acquires Apixio's Connected Care platform	Retrieval + AI coding consolidation	[47]
October 2024	Reveleer acquires Curation Health (prospective)	Retrieval vendors moving to prospective and point-of-care	[58] (secondary)
January 2025	Innovaccer $275M Series F (about $3.45B valuation); acquires Humbi AI	Capital still flowing to platform leaders	[46]
January 2025	Inovalon CEO appointment	Leadership change at a KLAS leader	[61]
March 2025	Cotiviti completes Edifecs acquisition (Edifecs "is now Cotiviti")	Incumbent consolidating interoperability and quality	[37][58]
April 2025	Reveleer acquires Novillus	Continued tuck-ins	[58] (secondary)
2025	Datavant acquires Aetion, Ontellus (record retrieval), DigitalOwl (AI record analysis)	Retrieval market roll-up; AI abstraction absorbed by scale players	[47]
July 2025	Arcadia majority investment by Nordic Capital (reported)	PE appetite for payer-provider data platforms	[58] (**not verified**)

Date	Event	Signal	Source
September 2025	Innovaccer acquires Story Health	Platform players diversifying beyond software	[46]
2025–2026	Cozeva–Vatica Health merger; MRO–Q-Centrix merger; Episource folded into Optum	Consolidation of quality, risk and point-of-care services	[51][52][53] [60]
September 2026	MRO launches Prodigy AI abstraction engine	AI abstraction becoming table stakes	[53]

Read-through. Capital and M&A are concentrating around (a) retrieval and AI abstraction roll-ups (Datavant, Reveleer, MRO), (b) platform players adding pharmacy, point-of-care and AI (Innovaccer, Cozeva, Arcadia), and (c) incumbents absorbing interoperability (Cotiviti–Edifecs). No verified shutdowns were found. Standalone point tools face a shrinking window as acquirers absorb them.

5. Gap Analysis and Differentiation

5.1 Unmet Needs

Unmet Need	Evidence	Who Is Underserved	Confidence (H/M/L)
A practical path from traditional to digital (FHIR/CQL) HEDIS, covering FHIR data mapping to the HEDIS Core IG, member-level reconciliation, and cut-over planning	NCQA digital-only about 2030, hybrid out by MY 2029 [9][11]; DCS requires FHIR data mapped to the HEDIS Core IG [13]; 85% of plans have not centralized data (vendor survey) [44]; vendor roadmaps say to "get a digital engine running early" [40]	Regional, Blue, provider-sponsored plans and Medicaid MCOs without large data-engineering teams	H
Year-round, audit-ready supplemental data "factory" to replace hybrid chart chase	Hybrid measures (GSD, CBP/BPD, PPC, TRC, COA) move to ECDS [9][10]; claims of 35–75x more supplemental data needed (vendor) [58]; abstraction is the scarce resource [39]	Plans that rely heavily on the hybrid season for diabetes, BP and perinatal measures, especially Medicaid	H
Explainable, member-level "why is this gap open?" logic that providers and auditors can trace	Vendors market transparency as a differentiator [36]; NCQA open-sourced CQL to reduce proprietary interpretation [12]; engine certification is required [11]	Quality analysts and provider-engagement teams who distrust black-box gap lists	M
Stars scenario modeling under methodology and legal volatility (measure removals, *Clover* recalculation, Tukey)	11 measures removed [5]; data-type restriction and recalculation [21]; lawsuits by Elevance, Alignment, SCAN [22][23]	Contracts clustered at 3.5 stars (175 contracts [4])	M
Multi-payer gap consolidation for provider groups, to reduce duplicate outreach and record requests	Duplicate record requests cause provider abrasion [26]; vendors pitch multi-payer execution layers [58]	IPAs, ACOs and medical groups in several payer VBC contracts	M
Affordable, fast-to-deploy tooling for small plans (<250K members)	Enterprise deployments are multi-quarter [58]; in-house build about $1.15M a year [59]	Small MA, SNP and Medicaid plans	L–M

5.2 Is the Gap Real? (Verdict)

Verdict: crowded market; partial gap by segment and capability.

The core HEDIS calculation is commoditized. About 80 organizations appear on NCQA's MY 2025 measure-certification status list [15]. Four or more vendors market NCQA-validated FHIR/CQL engines [39][41][51][58]. Customer satisfaction is so close that a different vendor won Best in KLAS in each of 2024, 2025 and 2026 [33][34][35]. Retrieval and AI-assisted abstraction are consolidating fast into well-funded players (Datavant, Reveleer, MRO/Q-Centrix, Cotiviti, Optum) [47][53][58][60].

What is NOT a real gap: - "An automated HEDIS measure calculator" (BC06's original framing). Every certified vendor already has one, and without NCQA certification it cannot be used for submission. - "In-year / real-time gap lists" and "quality dashboards". These are standard in Innovaccer, Arcadia, Cozeva, Cotiviti, Inovalon and others [43][45][51][36]. - "AI chart abstraction" on its own. Astrata, Reveleer, MRO, Datavant (DigitalOwl) and Cotiviti already offer it [38][39][47][53]. - "AI gap prioritization" as a standalone product. It is a feature, and incumbents can copy it quickly.

Where a gap genuinely remains: 1. **Segment:** mid-sized regional, Blue and provider-sponsored plans and Medicaid MCOs (roughly 100K–1M lives). They face the MY 2029 hybrid cut-off and digital-only HEDIS without the in-house FHIR engineering that national carriers have, and they are poorly served by multi-quarter enterprise deployments. 2. **Capability:** the **transition layer** between the incumbent certified engine and digital HEDIS. That means FHIR data readiness (reusing CMS-0057-F API pipelines), traditional-versus-digital member-level reconciliation, and a **human-in-the-loop, audit-traceable**

supplemental-data pipeline that turns year-round chart evidence into ECDS-eligible data. Vendors either sell engines (Smile, Astrata, the incumbents) or sell retrieval and abstraction (Datavant, Reveleer). Few sell the plan-specific data engineering and operating-model change that connects the two. 3. **Price point and delivery model:** a services-led accelerator that works alongside the plan's current certified vendor rather than replacing it. This avoids the rip-and-replace barrier and the certification cost.

This gap is real but limited in time, roughly 2026–2030. Incumbents will fill it as their digital engines mature. It is best pursued as a **systems-integration and accelerator offer**, not as a new HEDIS software product.

5.3 Differentiation Opportunity for KANINI

Where KANINI could credibly win - **Wedge: "Digital HEDIS Readiness + Prospective Evidence" program for mid-sized plans.** A fixed-scope engagement plus a reusable accelerator. It (1) maps the plan's claims, lab, EHR and supplemental feeds to FHIR per the HEDIS Core IG, (2) runs an open-source or partner CQL engine in parallel with the incumbent certified engine and reconciles results at member level, and (3) stands up an AI-assisted, abstractor-confirmed supplemental data pipeline for the measures leaving hybrid (GSD, CBP/BPD, PPC, TRC, COA). - **Dual-compliance positioning.** The same FHIR pipelines serve CMS-0057-F Provider Access and Payer-to-Payer APIs (due January 1, 2027) [17] and digital HEDIS [13]. One investment meets two mandates, which a CFO can approve more easily than a pure quality project. - **Partner, don't compete, on the engine.** Offer the approach with validated engines (e.g., NCQA Digital Content Services [13], Smile [41], Astrata [39]) or the plan's current vendor. KANINI's value is data engineering, Propel-accelerated build and testing, and operating-model change. - **Audit-first AI.** Position the AI layer on traceability (source passage, abstractor decision, lineage into supplemental files) rather than raw speed. This fits the regulatory stance and answers the main buyer objection. - **Stars scenario modeling as a follow-on.** Offer a contract-level scenario model (measure removals, *Clover* data-type restriction, Tukey/guardrail bands) to the 3.5-star cluster [4][21].

Barriers to entry - **Certification and audit.** Submission requires certified measure logic and passing a HEDIS Compliance Audit. KANINI should not certify its own engine without a committed anchor client. - **Data access.** PHI, BAAs, HITRUST/SOC 2 expectations, plus access to provider EHR and HIE data for supplemental evidence. - **Incumbent lock-in and the season calendar.** Engines are multi-year contracts, and changes cluster in summer and fall. - **Domain credibility.** Buyers expect HEDIS-literate staff (certified abstractors, HEDIS auditor experience) and references. - **Crowded messaging.** Every vendor already claims "AI + digital HEDIS". The differentiation has to show in delivery evidence, such as reconciliation results and chase-volume reduction, not in positioning.

5.4 Risks and Open Questions

Risk / Open Question	Why It Matters	How to Validate
Per-submission spend ($0.5M–$1.2M) behind the SAM estimate is an assumption	Drives the $1.0–2.0B market figure	8–10 buyer interviews (VP Quality at regional and Medicaid plans); review public state Medicaid MCO contracts and RFPs
Chart chase cost per chart ($15–$50) not verified	BC06 value case relies on it	Ask retrieval vendors and 3–5 plans for blended cost per retrieved and abstracted chart
BC06 "$4.5B quality measurement spend" not verified	Was used as a headline figure	Drop unless a primary source is found
NCQA digital-only date may slip ("depending on market readiness" [11])	Urgency of the wedge	Track NCQA Digital Quality Hub and MY 2028 spec releases
Clover ruling may be appealed or overturned; recalculation litigation (Elevance, Alignment, SCAN) ongoing	Changes HEDIS weight in Stars	Monitor court dockets and CMS 2028 Advance Notice
Incumbents' digital engines mature faster than expected	Closes the transition-layer gap	Quarterly check of NCQA certification list [15] and vendor dQM announcements

Risk / Open Question	Why It Matters	How to Validate
Auditor acceptance of AI-proposed, abstractor-confirmed supplemental data	Core of the AI value proposition	Walk through the design with an NCQA-licensed audit organization before building
Several competitor facts rely on secondary or vendor sources (Cotiviti–Edifecs date, Reveleer tuck-ins, Arcadia–Nordic, Cozeva–Vatica date, HealthEdge and Veradigm ownership status)	Accuracy of competitive positioning	Confirm via primary press releases before client use
KLAS segment scores and complaints are paywalled	Weakness evidence is thin	Obtain KLAS report access or run win/loss interviews
Labor scarcity for abstractors is asserted but not quantified	Supports AI productivity case	Use BLS OES data for medical records specialists and plan staffing interviews
HEDIS licence terms restrict use of NCQA specifications and value sets in commercial products	Demo and product IP risk	Confirm NCQA licensing for demos; use VSAC/eCQM public logic for demos (Appendix A)

Appendix A. Data Source and Setup

A.1 Primary Data Source

Synthea synthetic patient data (MITRE). Synthea is an open-source generator of realistic, longitudinal synthetic patients. It outputs FHIR R4 bundles, CSV tables, C-CDA and optional claims. Records use real terminologies (SNOMED CT conditions, LOINC labs and vitals, RxNorm medications, CVX immunizations, and ICD-10-CM / CPT / HCPCS on claims), so code-driven HEDIS logic behaves realistically.

- Main site: https://synthea.mitre.org/ · Downloads (pre-generated sets): https://synthea.mitre.org/downloads
- Source and wiki: https://github.com/synthetichealth/synthea · https://github.com/synthetichealth/synthea/wiki
- FHIR for Research overview: https://mitre.github.io/fhir-for-research/modules/synthea-overview

Relevant contents: `patients`, `encounters`, `conditions`, `observations` (HbA1c, BP, eGFR, uACR), `procedures` (mammography, colonoscopy), `medications`, `immunizations`, `claims` / `claims_transactions`, `payers` and `payer_transitions` (enrollment spans for continuous-enrollment logic), plus optional clinical notes for NLP/abstraction demos.

A.2 Data Generation / Acquisition Process

1. `git clone https://github.com/synthetichealth/synthea.git` and `cd synthea`.
2. Generate a reproducible population: `./run_synthea -p 100000 -s 12345 Massachusetts` (repeat for 2–3 states for geographic spread).
3. Key settings in `src/main/resources/synthea.properties`: - `exporter.fhir.export = true` and `exporter.csv.export = true` - `exporter.years_of_history = 10` (colonoscopy has a 10-year lookback) - `exporter.practitioner.fhir.export = true`, `exporter.hospital.fhir.export = true` (provider attribution) - `exporter.clinical_note.export = true` (unstructured notes for the AI abstraction layer) - `generate.only_alive_patients = false` (tests death and hospice exclusions)
4. Load FHIR bundles into a FHIR server (e.g., HAPI) for the digital path, and CSV into a relational store for the traditional path.
5. Download value sets from VSAC (UMLS licence) and CQL libraries for overlapping eCQMs from the eCQI Resource Center (A.4).

A.3 Use-Case-Specific Data Requirements

- **Population:** at least 100,000 members across 2–3 states with 3+ years of history for measure logic; a 10-year history for COL-E lookbacks.
- **Target measure set (current names, MY 2026):** GSD (Glycemic Status Assessment for Patients With Diabetes), EED (Eye Exam for Patients With Diabetes), BPD (Blood Pressure Control for Patients With Diabetes), KED (Kidney Health Evaluation for Patients With Diabetes), CBP (Controlling High Blood Pressure), BCS-E (Breast Cancer Screening), COL-E (Colorectal Cancer Screening), CCS-E (Cervical Cancer Screening), SPD (Statin Therapy for Patients With Diabetes), AAB (Avoidance of Antibiotic Treatment for Acute Bronchitis/Bronchiolitis). *Replaces BC06's list: CDC was split into GSD/EED/BPD/KED; ABA was retired.*
- **Validation of generated events:** confirm that Synthea produces HbA1c (LOINC 4548-4), systolic and diastolic BP (8480-6 / 8462-4), mammography, colonoscopy and FIT events, and retinal exams (EED coverage in Synthea may be thin; inject if needed).
- **Enrollment:** derive coverage spans from `payer_transitions` and inject 30–45-day gaps into 5–10% of members to test continuous-enrollment logic.

- **Injected test conditions:** (a) late-year BP readings that flip CBP compliance; (b) HbA1c values in 7.9–9.1% bands; (c) hospice and palliative-care exclusions; (d) evidence present *only* in clinical notes (e.g., historical colonoscopy text) to exercise AI abstraction plus abstractor review; (e) a simulated lab-feed outage to test anomaly detection.
- **Benchmarks:** load Star Ratings cut points by measure for the Stars impact modeler.

A.4 Additional Reliable Data Sources

Source (URL)	Contains	Use	Format / Access
NCQA HEDIS Measures and Technical Specifications — https://www.ncqa.org/hedis/measures/	Volume 2 specs (FHIR-aligned format from MY 2026), value set directory, annual changes	Authoritative measure logic for production accuracy	PDF/Excel; **licence purchase**
NCQA Digital HEDIS / Digital Content Services — https://www.ncqa.org/programs/data-and-information-technology/digital-hedis/	CQL measure packages, HEDIS Core FHIR IG (on request), digital reporting guidance	Digital (FHIR + CQL) path and parallel reconciliation	Licence; open-source engine requirements via Firely
NLM Value Set Authority Center (VSAC) — https://vsac.nlm.nih.gov/	Code lists (ICD-10, CPT, LOINC, RxNorm, SNOMED) for eCQMs	Value sets for demo measures that overlap eCQMs	Free with UMLS account (https://uts.nlm.nih.gov/)
CMS eCQI Resource Center, eCQMs and dQMs — https://ecqi.healthit.gov/	eCQM CQL/FHIR libraries, dQM strategy and education	Public reference logic (e.g., CMS122 diabetes HbA1c, CMS165 BP control, CMS130 colorectal screening) for demos	Free download
HL7 Da Vinci DEQM IG — https://hl7.org/fhir/us/davinci-deqm/	FHIR profiles for data exchange for quality measures and gaps-in-care reports	Standard format for provider gap reports and supplemental data exchange	Free
CMS Part C and D Performance Data / Star Ratings Technical Notes — https://www.cms.gov/medicare/health-drug-plans/part-c-d-performance-data	Measure cut points, weights, methodology, contract-level ratings	Stars impact modeler; benchmark targets	Free (PDF/Excel/CSV)
NCQA Quality Compass — https://www.ncqa.org/quality-compass/	National and regional HEDIS percentiles by product line	Realistic benchmark bands for rates	**Subscription**
Medicaid and CHIP Core Set resources — https://www.medicaid.gov/medicaid/quality-of-care/performance-measurement/adult-and-child-health-care-quality-measures	Core Set measure lists, technical specs, state performance data	Medicaid MCO and state reporting scenarios	Free
AHRQ Quality Indicators — https://qualityindicators.ahrq.gov/	PQIs and other indicators with SAS/SQL software	Supplementary utilization and quality measures; reference implementation patterns	Free
CMS-0057-F and CMS Interoperability resources — https://www.cms.gov/priorities/key-initiatives/burden-reduction/interoperability	API requirements and implementation guides	Designing data pipes that serve both the APIs and digital HEDIS	Free

Appendix B. Glossary of Terms

Term	Definition
HEDIS	Healthcare Effectiveness Data and Information Set: NCQA's set of 90+ health plan performance measures.
NCQA	National Committee for Quality Assurance: develops HEDIS, accredits plans, certifies measure vendors and licenses HEDIS auditors.
Measurement year / measurement period (MY)	The calendar period a measure evaluates. From MY 2026, NCQA uses "measurement period" in FHIR-aligned specs.
Initial population / denominator	Members eligible for a measure (age, diagnosis, continuous enrollment) after exclusions.
Numerator	Members in the denominator who meet the quality criterion (e.g., most recent BP <140/90).
Exclusion	Criteria that remove members from a denominator (e.g., hospice, palliative care, advanced illness and frailty).
Continuous enrollment	Requirement that a member is enrolled for a defined period (usually 12 months with one allowable gap of up to 45 days).
Care gap	A member in the denominator who does not (yet) meet the numerator.
Value set	A defined list of codes (ICD-10, CPT, LOINC, RxNorm, SNOMED) that qualifies an event for a measure component.
Administrative method	HEDIS reporting from claims, encounter, enrollment and pharmacy data.
Hybrid method	HEDIS reporting that combines administrative data with medical record review of a systematic sample. Being removed by MY 2029.
ECDS	Electronic Clinical Data Systems: NCQA reporting standard for structured electronic data (EHR, HIE/registry, case management, claims).
Supplemental data	Data beyond claims used to show numerator compliance or exclusions; "non-standard" supplemental data (e.g., abstracted chart evidence) requires audit proof.
Medical record review (MRR) / chart chase	Requesting and abstracting provider medical records to find evidence missing from claims.
Digital HEDIS / dQM	Digital quality measures: computable measure logic in CQL run against FHIR data.
FHIR	Fast Healthcare Interoperability Resources: HL7 standard for exchanging healthcare data via APIs (R4 is the regulated version).
CQL	Clinical Quality Language: HL7 standard language for computable measure logic.
HEDIS Core FHIR IG	NCQA's implementation guide defining how data must be represented in FHIR for digital HEDIS.
VSAC	NLM Value Set Authority Center, publisher of eCQM value sets.
eCQM	Electronic clinical quality measure used in CMS provider programs.
IDSS	NCQA's Interactive Data Submission System for HEDIS results.
HEDIS Compliance Audit	Required independent audit of a plan's HEDIS data and processes by an NCQA-licensed organization.
Star Ratings	CMS 1–5 star quality ratings for MA and Part D contracts.
QBP	Quality Bonus Payment: benchmark increase for MA contracts rated 4+ stars.
MA-PD	Medicare Advantage contract that includes Part D drug coverage.

Term	Definition
Cut points	Thresholds CMS sets each year that convert a measure score into 1–5 stars.
Tukey outlier deletion	Statistical method CMS uses to remove outlier contract scores before setting cut points (from the 2024 Star Ratings).
Guardrails	Caps on how far non-CAHPS cut points can move year to year.
Reward factor	Bonus added to the overall Star Rating for consistently high, low-variance performance.
EHO4All / HEI	Excellent Health Outcomes for All (formerly Health Equity Index) reward: finalized in 2024, cancelled in the CY 2027 final rule.
CAHPS	Consumer Assessment of Healthcare Providers and Systems: member experience survey used in Stars.
HOS	Health Outcomes Survey: Medicare member-reported outcomes survey used in Stars.
Core Set	CMS Child and Adult Core Sets of Medicaid/CHIP quality measures; mandatory reporting from 2024.
MCO	Managed care organization (here, a Medicaid managed care plan).
CMS-0057-F	CMS Interoperability and Prior Authorization Final Rule (January 2024).
Provider Access API / Payer-to-Payer API	FHIR APIs under CMS-0057-F for sharing member claims and clinical data with providers and with a member's new payer (due January 1, 2027).
USCDI	United States Core Data for Interoperability: standard set of clinical data elements.
TEFCA	Trusted Exchange Framework and Common Agreement: national health information exchange framework.
LOINC	Code system for lab tests and clinical observations.
CPT II	CPT Category II codes used to report quality data (e.g., BP ranges).
GSD, EED, BPD, KED	Diabetes measures that replaced the retired Comprehensive Diabetes Care composite: glycemic status, eye exam, BP control, kidney health evaluation.
CBP	Controlling High Blood Pressure measure.
COL-E / BCS-E / CCS-E	ECDS versions of colorectal, breast and cervical cancer screening measures.
Quality Compass	NCQA's subscription benchmarking database of HEDIS results.
KLAS	Healthcare IT research firm that publishes customer-reported vendor performance (Best in KLAS).
Synthea	Open-source synthetic patient data generator from MITRE.

Appendix C. References

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